

TickTock: Clinic-First Alerts for Stalled Home Delivery Orders

Product Requirements Document | October 7, 2026 | POC: Anvitha Abbagani

Independent case study, not an official Vetcove product. Try the prototype: ticktock.abbagani.com.

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Objective

A prescription can be approved by a vet, paid for by a pet owner, and still quietly get stuck. When that happens, the clinic may not know until the pet owner asks where their medication is.

Vetcove helps veterinary clinics manage purchasing and pharmacy operations. Its Home Delivery product lets clinics run their own online pharmacy, where vets approve prescriptions and pet owners order directly from the clinic instead of retailers like Chewy or Amazon.

The gap: once a prescription is approved, clinics have limited visibility into what happens next. When an order is delayed or stalled, the pet owner usually discovers the problem first, turning a fulfillment issue into a clinic experience problem.

TickTock closes that gap. It monitors approved orders against expected milestones and alerts clinics when something falls behind, prioritizing the most time-sensitive cases.

The goal: turn an owner-discovered problem into a clinic-managed workflow.

The Problem

Problem	R	I	C	E	RICE
P1: Pet owners hit stuck orders, with no clear status and no one to call	4	5	3	5	12
P2: After the clinic approves an order, nothing tells the clinic when it gets stuck	4	4	3	2	24
P3: What reaches the patient record depends on the clinic's practice software	3	3	4	4	9

P1 is the problem everyone sees, but it is a symptom. Orders get stuck for many reasons: shortages, slow compounding, shipping. Those are operations problems, and they can't be seen from outside. What a product can change is what happens next. Today, a stuck order can turn into 12 days of silence that the pet owner discovers first. That is P2.

- Vetcove already watches for stuck orders, but only for the clinic's own supplies. It has a backorder dashboard and back-in-stock alerts ([backorders](#), [stock alerts](#)).
- No similar alerts is sent to the clinic when a client's approved order gets stuck.
- P3 makes it worse. "Practice software" (also called a PIMS) is the system clinics use for patient records. Two popular ones, Digitail and Shepherd, sync with Vetcove only once a night, and cancellations don't come through at all. So a clinic that checks its own records may see old information ([Digitail](#), [Shepherd](#)).

Solving P2 comes first. A more accurate patient record (P3) doesn't help if nobody is told to look at it. Full evidence is in Appendix A.

Who This Is For

Primary: the practice manager who runs Home Delivery at an independent clinic

- They own the clinic's online store, and they take the phone calls when it goes wrong. Today they learn about a stuck order from the pet owner.
- They want one list of stuck orders: most urgent first, with who is handling each one.

Secondary: the veterinarian who approves prescriptions

- They approve Home Delivery orders and are responsible if a patient runs out.
- They want critical drugs (insulin, seizure, cancer and heart medications) to rise to the top without reading every row.

The pet owner benefits but never uses TickTock. What they see is a clinic that calls them first.

The Solution: TickTock

Vetcove already watches for backorders on clinics' own supply orders. TickTock points to the same idea at the pet owner's order, after the clinic approves it.

The core

- Deadlines. Every approved order should be confirmed, shipped, and moved on time. Compounded (custom) medicines get more time as they take longer ([Captterra, Jun 2026](#)).
- Five rules spot a stuck order: R1 not confirmed, R2 not shipped, R3 stuck in transit (no tracking update for more than 3 days), R4 backordered or cancelled, R5 autoship missed.
- An alert the clinic owns. It says what's wrong in plain words and how late it is, then moves Stuck → Waiting → Done. TickTock records who did what, and when. No AI makes decisions.

What makes it different

1. Most urgent first (D1). The list is sorted by when the pet runs out (last fill + quantity ÷ daily dose), with critical drugs first. The clinic decides what counts as critical. Example: insulin 1 day late with 1 day left ranks above a flea treatment 6 days late with 47 days left. Sorting by lateness would get this backwards.
2. Suggested next step (D2). Each alert suggests the fastest fix for that problem:

Problem	Suggested fix
Critical medicine with 3 days or less (any rule)	Offer pickup at the clinic today, with a drafted text to the owner
R1 Not confirmed	Nudge the pharmacy
R2 Not shipped	Wait for a new date (estimate pre-filled; "I've told the owner")

R3 Stuck in transit	Ask the pharmacy to reship
R4 Backordered	Switch to an in-stock alternative (a vet approves)
R5 Autoship missed	Send this delivery now

3. Nothing is sent or changed without a clinic user confirming. Details are in Appendix C.
4. Warning before approval (D3, next phase). Vetcove already sees live supplier stock for clinic purchasing (shop.vetcove.com), so it could warn a vet before approving: "Backordered at the pharmacy. The pet has about 4 days left."

What's not new: Order lists, assigning work, activity history, tracking emails, refill reminders, autoship and client texting already exist in Vetcove or its competitors, and delivery-delay alerts are common in online retail. TickTock's claim is: send the alert to the clinic; rank it by how urgent the medication is.

The prototype builds the core, D1 and D2. D3 is the next phase. Rejected ideas are in Appendix B.

How We'll Know It's Working

Key metric: Clinic-First Catch Rate. Of all stuck orders, the share the clinic picked up before the pet owner got in touch.

At 30 days, check two things:

- Time to Clinic Action: if alerts sit untouched, the problem is who owns them, not the rules.
- False Alarm Rate: if many "stuck" orders arrive on time anyway, the deadlines are too tight and need loosening.

At 60 days, the Clinic-First Catch Rate should go up, while the Ignored Alert Rate stays flat. If both go up together, clinics are catching stalls only because they're flooded with alerts. The Before-Run-Out Rate should be clearly higher for critical drugs than for standard ones. That shows D1's ranking changes what clinics do.

At 90 days, report Where-Is-My-Order Contacts per 100 Orders to leadership.

- If it drops, the idea works.
- If clinics catch stalls first but contacts don't drop, check whether clinics act on alerts (D2) before calling the idea wrong.

Each differentiator has its own metric: D1 → Before-Run-Out Rate · D2 → Suggestion Use Rate · D3 → Approval Warning Accuracy. The full table is in Appendix A.

Timeline

Task	Date
Ask: does this already exist? (three questions under Top Risks)	Day 0
Check the data: which order events are recorded (pharmacy confirmation, ship scan, tracking scans, daily dose)	+7 days
Test on history: run 90 days of past orders through R1–R5; count real stalls and false alarms	+14 days
Improve the prototype with 2–3 Home Delivery clinics	+21 days

Top Risks

- This may already exist behind Vetcove's login.
 - What to do: ask, don't assume. "When an approved order gets stuck at the pharmacy, how does the clinic find out today?" "Who handles a stuck order: the clinic or Vetcove support?" "Does Home Delivery use the same suppliers as clinic purchasing?"
 - If it exists, focus the pitch on what it doesn't do yet: ranking, next steps, early warning.
- Too many alerts. Clinics may start ignoring them.
 - What to do: set deadlines separately for each order type, put the most urgent first, and track ignored alerts and weekly volume from day one.
- Some order data may be missing, such as pharmacy confirmations or a daily dose.
 - What to do: when the dose is unknown, rank by lateness and show "supply unknown." Never guess a run-out date.

The full risk table is in Appendix E.

What's Next

- First: run past orders through TickTock to see what it would have caught, and how often it would have raised a false alarm. Do this before building anything.
- Then: a pilot of the core, D1 and D2 with volunteer clinics. The pilot shows which suggested fixes clinics actually accept, and which they replace with something else.
- D3 (early warning) depends on whether Home Delivery uses same suppliers as purchasing.
- Later: add alert notes to the patient record. This depends on P3. It's easiest first in Vetcove's own practice software, which launched an iPhone app in November 2025 ([App Store](#)).

Full detail is in Appendix F.

Appendix

A. Evidence & Metrics

P1: Pet owners hit stuck orders, with no clear status and no one to call

What the pet owner needs: "When my pet's medication is due, I need it to arrive, or someone to tell me what to do."

Evidence:

- Trustpilot ([source](#)): a score of 1.8 out of 5, from 15 reviews, all 1-star, dated January 28 to September 28, 2026. Common complaints:
 - no contact for 12 days about a cancer medication (Jul 20);
 - a week's delay on a compounded Cushing's medication (Aug 4);
 - a package in transit for 10+ days (Sep 10);
 - a late autoship of an allergy diet (Jan 28);
 - support that was AI-only (Aug 24, Aug 27).
- Capterra ([source](#)): 13 reviews, split cleanly by who wrote them.
 - All 10 five-star reviews come from clinic staff, mostly praising purchasing.
 - All 3 one-star reviews come from pet owners, and all are about Home Delivery.
 - Customer service scores 3.5, versus 4.3 for ease of use.
 - The pet-owner complaints:
 - 12 days with no update or reply (Jul 20, 2026);
 - more than 20 unanswered calls in 7 days (Apr 16, 2026);
 - a prescription cat food that took a week and a half to arrive, so the cat ran out (Aug 2, 2025).
- The delivery promise may leave out processing time. The Aug 2025 reviewer says the "1–5 day" estimate covered shipping only. Processing took another 5 business days. My inference: pet owners may be planning around a date that was never realistic.

Limits of this evidence:

- Only 15 Trustpilot reviews and 3 negative Capterra reviews, and unhappy customers are the ones who post.
- Vetcove says Home Delivery is "trusted by thousands of clinics."
- Some delays are likely drug shortages, which no software can prevent.

Why it matters: clinics' share of dog prescription sales fell 6.1 points from 2023 to 2024 (AVMA data via [Today's Veterinary Business, Sep 3, 2026](#)). A missed medication gives a client a direct reason to buy elsewhere.

P2: After the clinic approves an order, nothing tells the clinic when it gets stuck

What the clinic needs: "When an order with my clinic's name on it goes wrong, I want to know before my client does, so I can fix it."

Evidence:

- The store carries the clinic's name: "Branded storefronts" ([ezyVet](#)).
- Vets sent clients there. At least four Trustpilot reviewers say their vet moved them from Chewy or PetMeds (Jul 10, May 29, Aug 27). Clinics stepped in only after the problem (Sep 11, Sep 28). A Capterra reviewer says their vet began requiring Home Delivery.
- One order, step by step (Capterra, Aug 2, 2025):
 - the vet approved it on Monday, July 28;
 - it didn't ship until Friday evening, after the owner emailed to ask;
 - the email got no reply;
 - the cat ran out of its prescribed food, the clinic sold the owner food at a higher price;
 - the clinic now charges a fee to send prescriptions to Chewy instead.
- My inference: nothing told the clinic its approved order sat for four days. The clinic paid for it in trust and in lost pharmacy sales.
- Vetcove watches for stuck orders, but only for the clinic's own supplies. It has a backorder dashboard with supplier delivery dates ([help](#)). It also sends email or text alerts when items are back in stock ([help](#)).
- No public page describes a clinic alert for a stuck client order. I checked Vetcove's help center and the partner guides for ezyVet, Shepherd and Digitail.

Limits of this evidence: Vetcove's Home Delivery guides are behind a login. "Not found" is not the same as "doesn't exist." This is the first Top Risk.

P3: What reaches the patient record depends on the clinic's practice software

What the vet needs: "When I open a patient's chart, I want to see what was ordered, changed or cancelled, and who prescribed it."

Evidence:

- Digitail: syncs once a night. It "does not support multiple staff members," so every prescription shows the same default vet ([source](#)).
- Shepherd (Apr 11, 2025): syncs once a night. Prescriptions and orders come through, but "order modifications and cancellations do not update automatically" ([source](#)).
- ezyVet: refills, cancellations and order status update "in real time" .
- Pulse (Covetrus's practice software): a clinic reviewer says orders import with a default time of 12am, which makes client follow-up confusing ([Capterra, Jun 2026](#)).
- Industry context: a Covetrus survey (around 2022) found that online-pharmacy prescription requests take about 1.4 staff-hours a day. 71% of vets said those requests don't update the patient record. Covetrus sells competing software, and I saw this only in search summaries.

Why it matters for P2: if the record updates only at night, it can't warn staff that an order is stuck today.

Success metrics

Type	Metric	What it measures	Why it matters
Key	Clinic-First Catch Rate	% of stuck orders the clinic picked up before the pet owner got in touch	Measures P2 directly: who hears first
Secondary	Time to Clinic Action	Typical time from alert to the first action (assign or fix), and to Done	Shows whether clinics actually work the list
Secondary (D1)	Before-Run-Out Rate	% of critical-drug alerts closed before the pet runs out	Tracks the urgency ranking
Secondary (D2)	Suggestion Use Rate	% of alerts closed using the suggested next step	Tracks the one-tap next step
Secondary (D3)	Approval Warning Accuracy	% of early warnings where the order really did get stuck	Tracks the warning before approval
Secondary	Where-Is-My-Order Contacts per 100 Orders	How often pet owners ask about order status	The P1 symptom, measured downstream

Guardrail metrics (to catch side effects)

Type	Metric	What it measures	Why it matters
Guardrail	False Alarm Rate	% of alerts closed as "it arrived on time anyway"	Rules are too sensitive
Guardrail	Ignored Alert Rate	% of alerts closed without any action	Clinics are tuning alerts out
Guardrail	Alerts per Clinic per Week	How many alerts each clinic gets	Too many and the list stops being useful
Guardrail	Medication Changes After a Suggestion	Prescriptions changed after a D2 suggestion	Makes sure TickTock isn't nudging vets to switch drugs too often

B. Choosing the Solution

What already exists, and why it isn't enough

Overall, clinics have a positive impression of Vetcove (every five-star Capterra review is from clinic staff). Vetcove also has most of the pieces: supply-side backorder alerts, real-time order updates to

ezyVet, and one place for orders and client messages. What's missing publicly is the link between them: a pet owner's stuck order, turned into an alert the clinic owns.

Competitors:

- Vetsource advertises lifecycle emails, a dedicated account manager, pharmacist review and about 3-day average delivery ([source](#)). It doesn't describe stuck-order alerts for clinics.
- Covetrus vRxPro advertises automatic reminders to pet owners.

Both companies' clinic portals are behind a login, so this means not found, not "doesn't exist."

Scoring the options (RICE)

Option	R	I	C	E	RICE
S1 Core: deadline rules → alert list with history	4	4	4	2	32
S2 (D1) Most urgent first	4	4	3	2	24
S3 (D2) One-tap next step + client message draft	4	3	3	3	12
S4 (D3) Warning before approval, using supplier stock	3	4	2	3	8
S5 Alert notes added to the patient record	3	2	3	4	4.5

Why this order

- S1 and S2 have the highest confidence for the least effort. Everything else depends on them, and they are what the prototype builds.
- S3 turns an alert into a fix. It comes once we see what clinics actually do.
- S4 scores lowest of the three differentiators because of one unknown. But it's the only idea that uses something Vetcove has and a standalone pharmacy doesn't.
- S5 waits on P3.

Not in this phase

- Fixing why orders get stuck (pharmacy operations, shipping, shortages).
- What pet owners see when they track an order.
- Rebuilding how practice software syncs (P3).
- Pricing and fees.

Ideas considered and rejected

- Predicting delays with machine learning. There's no data to train or test it on from outside, and clinics need to know why something was flagged. Use rules first. Consider a model only after there's a history of real alerts.

- Swapping medications automatically. Only a vet can change a prescription. TickTock can suggest, never swap.
- Messaging pet owners automatically. The store carries the clinic's name, so a message about a medical delay should come from the clinic. TickTock drafts it; the clinic sends it.
- Moving orders to another pharmacy automatically. State pharmacy licenses and prescription transfer rules make this a human decision.
- Building it inside each practice software. It would inherit P3's inconsistency. It belongs in Vetcove's clinic portal, with patient-record notes added later (S5).

C. Prototype Walkthrough

This describes the prototype as of October 7, 2026, and only what it actually does.

- What it is: one offline web page; no login, server or internet connection. Texts and requests are simulated; nothing is sent.
- Data: a made-up clinic with 20 made-up orders, 7 of them stuck. The dates follow the current week, so the story never changes: Maple is 1 day late with 1 day of insulin left.
- The prototype is live on ticktock.abbagani.com; Signed in as Jane Doe, practice manager.

Walkthrough

1. One list. A 3-step strip explains the idea. The headline reads "7 of 20 orders are stuck." Each card shows the pet and medication, how much is left, what's wrong, who's on it, and a Fix button. Red and amber mean a critical medicine with 3 days or less, or 4–7 days; green means not urgent; grey means TickTock can't tell. Search finds any order.
2. Most urgent first (D1). Sorted by who runs out first, Maple's insulin is #1 and the flea treatment #7. "Sort by most overdue" puts the flea treatment at #1 and the insulin at #5. Making the Cushing's capsules critical moves Pepper from #6 to #4.
3. Fix with the suggested step (D2). Fix opens "Maple's insulin hasn't shipped · 1 day of insulin left" with a green Suggested box: offer pickup, with a drafted text and a note that clinic stock shows insulin on hand. Other options list the remaining fixes plus "It arrived" and "No longer needed." Edge cases you can see: Otis (heart medication not stocked → reship), Mochi (controlled substance; unreadable dose → "ask the owner how much is left"; shares an owner with Clementine, so one text covers both), Juniper (no refills → a vet renews first), and after 6 pm (pickup moves to tomorrow morning). The full list is in Appendix D.
4. Waiting and Done. Offering pickup moves the order to Waiting · owner to pick up. "Picked up ✓" closes it; "Owner said no" sends it back, and TickTock suggests the next best fix. "Wait for a new date" pre-fills an estimate, warns if it falls after the run-out date, and records whether the owner was told. "Show details" shows the timeline and a history of who did what, and when.
5. Two staff, one order. Otis is assigned to Sam K. The panel says so and stays locked until Jane taps Take over, so two people don't contact the same owner.

The "How it works" page fits on one screen: four steps, then each rule R1–R5 beside its suggested fix.

What the prototype must do (acceptance checklist). Suggestion rules are in the Solution table and edge-case behavior is in Appendix D; beyond those:

- One alert per stuck order, named after the first rule it broke.
- Sort in three groups: critical with 7 days or less (soonest run-out first), then "supply unknown" (most overdue first), then everything else (soonest run-out first). Ties go to the more overdue order. "Most overdue" sorts by lateness only.
- Waiting requires a date; estimates skip weekends and holidays.
- The list, counts and history update instantly after any search, tier change, assignment or fix.

Left out on purpose: a list of healthy orders; watching time pass; the warning before approval (D3); notes in the patient record (S5); real notifications, logins, saved data and integrations. The deadlines are demo placeholders.

Screens

#	Screen	What it shows	Step
1	Stuck orders	3-step strip; "7 of 20 orders are stuck"; 7 cards; insulin is #1	1
2	Sorted by most overdue	Same 7; flea treatment #1, insulin #5	2 (D1)
3	Fix panel	"Maple's insulin hasn't shipped," 1 day left, Suggested: offer pickup with drafted text	3 (D2)
4	After fixes	Fewer stuck; "Waiting (2)" with dates; "Done (1)"; details show the history	3
5	How it works	Four steps and the five rules	—

D. User Stories & Edge Cases

The full product, beyond the prototype.

User stories

- As a practice manager, I want to hear about stuck orders before the pet owner does, so I can call them first.
- As a vet, I want critical drugs sorted by run-out date, so a dog with one day of insulin isn't stuck behind a flea treatment.
- As a practice manager, I want a suggested next step on each alert, so fixing it takes a tap, not a phone tree. (D2)
- As a vet, I want to know before I approve if the item is backordered, so I can offer pickup before the client waits. (D3)

Requirements

- Clinics can set deadlines for each order type. The defaults come from testing on past orders.
- Every alert names its rule. No alert appears without a one-sentence reason.
- Critical and Standard come with sensible defaults, and the clinic can change any drug.
- Suggested next steps never change a prescription or message a client without a clinic user confirming. Medication swaps need a vet. (D2)
- A warning before approval shows where its stock information came from, and when. (D3)
- Critical alerts that nobody claims are escalated after a set time.

Edge cases

Every case has a decided behavior. Prototype = demonstrated in the prototype. Rule = defined for the full product; it needs time to pass or a real integration, so the prototype can't show it. Out of scope = deliberately not TickTock's job.

Case	What TickTock does
Dose can't be read	Shows "supply unknown"; for critical medicines, asks the owner how much is left
Pet already ran out	"Ran out N days ago," ranked first in its group
Medicine not in clinic stock	Doesn't suggest pickup; says why
Clinic closed (after 6 pm)	Offers pickup tomorrow morning
Owner says no to pickup	Back to stuck; suggests the next best fix
One owner, several stuck orders	One text covers all their pets
Two staff on one order	Second person must take over first
Controlled substance	Notes that only a vet can approve reship, switch or handover
No refills left	A vet renews first
Compounded order	Longer deadlines
Holidays	Skipped in estimated dates
Order no longer needed	Closes as "No longer needed," not a false alarm
Date in the past	Not allowed

Rules for the full product (need time to pass or real integrations)

Case	What TickTock does
Owner doesn't answer the pickup text	Back to stuck at closing time: "call them"
Waiting date passes	Back to stuck, tagged "Date passed"
Order arrives after an alert	Closes as "Delivered"; a false alarm only if it was on time
One pet, two stuck orders	Two alerts, one text
Clinic cancels / pharmacy cancels	No alert / R4 fires
Client skips an autoship on purpose	No alert
Holidays in deadline checks	Skipped

Out of scope: wrong item, damaged package or refund disputes. They don't break a deadline; Vetcove's support process owns them.

E. Event Tracking & Risks

Events to track

Event	What it records	Feeds
Order approved	An order starts being tracked	The base count for every rate
Alert raised	A rule fires (rule, tier, days left)	Alerts per Clinic per Week, Clinic-First Catch Rate
Alert assigned	Who took it	Time to Clinic Action
Owner contacted	The clinic texts the owner or ticks "I've told the owner"	Clinic-First Catch Rate
Client asked	The owner asks about the order (text, call, ticket)	Clinic-First Catch Rate, Where-Is-My-Order Contacts
Fix used	Which suggested fix, and whether it led to Waiting or Done	Suggestion Use Rate, Medication Changes After a Suggestion
Pickup declined	The owner says no to pickup	Suggestion Use Rate
Waiting date passed	The order goes back to stuck	False Alarm Rate, Time to Clinic Action

Alert closed	Arrived, fixed or no longer needed	Before-Run-Out, False Alarm and Ignored Alert rates
Approval warning shown (D3)	A warning appears at approval	Approval Warning Accuracy

Assumptions & risks

We assume	If wrong, we...	Act when
It isn't already built in Vetcove's portal	Refocus the pitch on D1–D3	The three Top Risks questions say it exists
Order events are recorded	Launch only the rules with data behind them	Fewer than 2 of R1–R3 have data at the +7-day check
Deadlines can be tuned per order type	Loosen deadlines	False Alarm Rate over 25% at 30 days
Clinics will own their alerts	Route by role, send a daily summary, escalate critical ones	Typical response over 1 business day at 30 days
The daily dose is usually recorded	Ask for the dose at approval	Over 30% of alerts show "supply unknown"

F. Next Steps

Open questions (plus the three under Top Risks)

- How often is the daily dose recorded? D1 depends on it.
- Can TickTock see the clinic's stock?
- Which controlled-substance fixes need a vet, or aren't allowed?

To build: a scheduled deadline check, an editable Critical drug list, and alert assignment.

To reuse: order status events (with timestamps added), inventory counts, and staff accounts.